Los Angeles County, California

Prioritize targeted follow-up

Gap review score **85.6** / 100

Why it surfaced

Los Angeles County has a published gap review score of 85.6, placing it in the Prioritize targeted follow-up category. This score is not a measure of individual risk.

County data: Six review factors

These factors summarize the published data used in the Atlas. They do not show how much each factor contributed to the county's overall score.

1	**Published Lyme case data**	The published 2023 county case-count floor is 15. The minimum rate supported by that floor was 0.1 cases per 100,000. That floor is not a complete case count or incidence estimate. 43 published 2023 human-surveillance records were not allocated to a county in this release. Check whether any belong in this county before interpreting the county figure.
2	**Tick presence**	Published tick surveillance classifies this county as established. *Ixodes pacificus* is established. A published status is not a measure of tick abundance.
3	***Borrelia burgdorferi* detection**	*Borrelia burgdorferi* was detected in tested ticks in the published surveillance data. Detection is not a human infection or transmission estimate.
4	**Social vulnerability**	The 2022 social vulnerability percentile is 86.3. Higher values indicate greater vulnerability on the source index. This is context for access and outreach, not proof of underreporting.
5	**Health insurance coverage**	The published percentage uninsured is 9.0%. This is context for healthcare access, not proof that cases are being missed.
6	**Rurality**	USDA Rural-Urban Continuum Code 1 (2023): metro area of 1 million or more. Code 1 is the most metropolitan and 9 is the most rural. This is context for follow-up planning, not a tick-habitat or underreporting measure.

Interpreting the data

Lyme disease data are grouped by county of residence, not necessarily the location of exposure. These values describe published surveillance, not individual risk. The percentage uninsured is context; the score uses the uninsured percentile.

Observation periods: Lyme case records 2023; social vulnerability 2022; rurality 2023.

Suggested follow-up

Consider giving greater attention to focused, locally governed review when the published response category is strongest.

1	**Human disease surveillance**	Consider prioritizing focused review of locally available Lyme disease surveillance information. Compare the published county case-count floor with state or local surveillance records, including reporting completeness and residence-versus-exposure differences where investigation notes already exist.
2	**Vector and pathogen surveillance**	Consider prioritizing review with vector surveillance partners of the relevant tick and pathogen findings. Review the published tick presence and *Borrelia burgdorferi* detection records, including the time and geographic coverage of collections where local records include that detail. Detection is not a human transmission estimate.
3	**Clinical and laboratory reporting**	Where local findings indicate a meaningful case-ascertainment concern, consider focused discussion with relevant surveillance or laboratory-reporting partners. Compare reporting pathways with the published human and pathogen records. This county record does not indicate a test for any individual patient.
4	**Community context and outreach**	Consider whether locally verified information supports focused prevention education or outreach. The review category alone does not justify a public warning. Use social vulnerability, health insurance coverage, and rural-urban classification as context for access and outreach. These measures are not proof of transmission or underreporting.
5	**Coordination and documentation**	Consider prioritizing a defined follow-up question using existing jurisdictional decision processes. Document supporting data, remaining uncertainties, and local criteria for proceeding or stepping back.

Questions before acting

1. What verified evidence makes this county a stronger candidate for focused follow-up than other current program priorities?
2. Have alternative explanations—including travel-associated exposure, surveillance limitations, and reporting practices—been considered?
3. What specific action could reduce important uncertainty or improve a locally supported public-health response?
4. What additional evidence, operational capacity, and local approval would be needed before proceeding?

Data sources and more information

Definitions, data sources, and methods are available on the Atlas website.

Lyme surveillance data | Tick and pathogen data | Clinical testing guidance | Social vulnerability index | Health insurance data | Rural-urban classification | County review methodology

Document details

County: Los Angeles County, California
County identifier (FIPS): 06037
Published gap review score: 85.6
Review category: Prioritize targeted follow-up

Data reference years

Lyme case records: 2023
Social vulnerability: 2022
Rurality: 2023

Document information

Release date: 2026-10-20
Template version: county data brief v3.0
Website: onehealthatlas.org
Data release: governed-2026-10-15-january-climate-prod

Data methodology: semantic-2.0.0
Score methodology: surveillance-gap-score-v1.1; smooth k=50
Generated: 2026-10-20 18:00 UTC
Source vintages: MRLC Annual NLCD 2025 reviewed demo cohort (C1V2-2025); USDA ERS Rural-Urban Continuum Codes (2023); CDC/ATSDR Social Vulnerability Index (2022 (2018-2022 ACS)); CDC Lyme surveillance (2023); CDC ArboNET tickborne pathogen status (through 2025-12-31); CDC ArboNET county tick status (through 2025-12-31)
Published release only. What-if score settings are not applied.
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